

DYNAFLEX

	DESCRIPTION	CONTENT
DYNAFLEX TABLET 275MG MAL05051065AZ	TABLET Colour : Light Blue Shape : Oval and Biconvex	Each Tablet Contains: Naproxen Sodium 275 mg Equivalent To Naproxen 250 mg
DYNAFLEX TABLET 550MG MAL19950335AZ	TABLET Colour : White Shape : Oblong	Each Tablet Contains: Naproxen Sodium 550 mg Equivalent To Naproxen 500 mg

PHARMACODYNAMICS:

Naproxen is a non-steroidal anti-inflammatory analgesic compound with antipyretic properties as has been demonstrated in classical animal test systems. Naproxen exhibits its anti-inflammatory effect even in adrenalectomised animals, indicating that its action is not mediated through the pituitary-adrenal axis. Naproxen inhibits prostaglandin synthetase (as do other NSAIDs). As with other NSAIDs, however, the exact mechanism of its anti-inflammatory action is not known.

PHARMACOKINETICS:

Naproxen is completely absorbed from the gastrointestinal tract, and peak plasma levels are reached in 2 to 4 hours. Naproxen is present in the blood mainly as unchanged drug, extensively bound to plasma proteins. The plasma half-life is between 12 and 15 hours, enabling a steady state to be achieved within 3 days of initiation of therapy on a twice daily dose regimen. The degree of absorption is not significantly affected by either foods or most antacids. Excretion is almost entirely via the urine, mainly as conjugated Naproxen, with some unchanged drug. Metabolism in children is similar to that in adults. Chronic alcoholic liver disease reduces the total plasma concentration of Naproxen but the concentration of unbound Naproxen increases. In the elderly, the unbound plasma concentration of Naproxen is increased although total plasma concentration is unchanged.

INDICATION:

For rheumatic disorders such as ankylosing spondylitis, osteoarthritis and rheumatoid arthritis. Mild to moderate pain such as dysmenorrhoea, migraine and some musculoskeletal disorders and in acute gout.

RECOMMENDED DOSE:

Take the medication orally, with or after meals.

Dysmenorrhoea, pain following IUD insertion, acute musculoskeletal pain: Initially 550 mg then 275 mg three times daily. The daily dose should not exceed 1375 mg.

Post-partum pain: Single dose of 550 mg.

Migraine treatment: 825 mg at onset then 275-550 mg later in the day. The daily dose should not exceed 1375 mg.

Migraine prophylaxis: 550 mg twice daily.

Chronic musculoskeletal pain: 275-550 mg twice daily.

Paediatric use: Not recommended for use in children under 16 years of age.

There is a greater risk of NSAID-induced gastrointestinal bleeding in the elderly. It is therefore recommended that patients in this age group, especially those 70 years of age or older, be given half the usual adult dose initially.

After assessing the risk/benefit ratio in each individual patient, the lowest effective dose for the shortest possible duration should be used.

CONTRAINDICATIONS:

Should not be given to patients hypersensitive to Aspirin and Naproxen. Should not be given to patients in whom Aspirin or other NSAIDs induce asthma, rhinitis or urticaria. Active peptic ulceration.

PRECAUTIONS:

Naproxen should be given with care to patients with bleeding disorders, cardiovascular disease, peptic ulceration or a history of such ulceration, and in those who are receiving coumarin anticoagulants. Paediatric use: not recommended for use in children under 16 years of age.

CARDIOVASCULAR THROMBOTIC EVENTS

Observational studies have indicated that non-selective NSAIDs may be associated with an increased risk of serious cardiovascular events, principally myocardial infarction, which may increase with dose or duration of use. Patients with cardiovascular disease or cardiovascular risk of an adverse cardiovascular event in patient taking NSAID, especially in those with cardiovascular risk factors, the lowest effective dose should be used for the shortest possible duration.

There is no consistent evidence that the concurrent use of Aspirin mitigates the possible increased risk of serious cardiovascular thrombotic events associated with NSAID use.

HYPERTENSION

NSAIDs may lead to the onset of new hypertension or worsening the pre-existing hypertension and patients taking antihypertensive with NSAIDs may have an impaired antihypertensive response. Caution is advised when prescribing NSAIDs to patients with hypertension. Blood pressure should be monitored closely during initiation of NSAID treatment and at regular intervals thereafter.

HEART FAILURE

Fluid retention and oedema have been observed in some patients taking NSAIDs, therefore caution is advised in patients with fluid retention or heart failure.

GASTROINTESTINAL EVENTS

All NSAIDs can cause gastrointestinal discomfort and rarely serious, potentially fatal gastrointestinal effects such as ulcers, bleeding and perforation which may increase with dose or duration of use, but can occur at any time without warning. Caution is advised in patients with risk factors for gastrointestinal events e.g. the elderly, those with a history of serious gastrointestinal events, smoking and alcoholism. When gastrointestinal bleeding or ulceration occur in patients receiving NSAIDs, the drug should be withdrawn immediately. Doctors should warn patient about signs and symptoms of serious gastrointestinal toxicity. The concurrent use of Aspirin and NSAIDs also increases the risk of serious gastrointestinal adverse events.

SEVERE SKIN REACTIONS

NSAIDs may very rarely cause serious cutaneous adverse events such as exfoliative dermatitis, toxic epidermal necrolysis (TEN) and Steven-Johnson Syndrome (SJS), which can be fatal and occur without warning. These serious adverse events are idiosyncratic and are independent of dose or duration of use. Patients should be advised of the signs and symptoms of serious skin reactions and to consult their doctor at the first appearance of a skin rash or any other sign of hypersensitivity.

WARNINGS

RISK OF GI ULCERATION, BLEEDING AND PERFORATION WITH NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

DRUG INTERACTIONS:

Due to high plasma-protein binding of Naproxen, patients simultaneously receiving hydantoins should be closely monitored for adjustment of dose if required. Interactions have not been observed in clinical studies with Naproxen Sodium and anticoagulants or sulfonylureas, but caution is advised, since interaction has been seen with other non-steroidal agents of this class. The natriuretic effect of Furosemide has been reported to be inhibited by some drugs of this class. Inhibition of renal Lithium clearance leading to an increase in plasma Lithium concentration has also been reported. Naproxen Sodium can reduce the antihypersensitive effect of Propanolol and other β -blockers. Probenecid given concurrently increases Naproxen plasma levels and extends its plasma half-life significantly.

PREGNANCY AND LACTATION:

Good medical practice indicates minimal drug usage in pregnancy, and use of this class of therapeutic agent requires cautious balancing of possible benefit against potential risk to the mother and foetus especially in the first and third trimesters. The use of these products should be avoided in patients who are breast feeding.

SIDE EFFECTS/ADVERSE REACTIONS:

Gastrointestinal disturbances. Peptic ulceration and gastrointestinal bleeding. Headache, dizziness, nervousness pruritus, tinnitus, oedema, depression, drowsiness, insomnia, blurred vision and other ocular reactions. Hypersensitivity reactions, abnormalities of liver function test, impairment of renal function, agranulocytosis and thrombocytopenia have occasionally been observed. Dermatological hypersensitivity: skin rash, urticaria, angio-oedema. Anaphylactic reactions to Naproxen and Naproxen Sodium formulations, eosinophilic pneumonitis, alopecia, erythema multiforme. Steven-Johnson Syndrome, epidermal necrolysis and photosensitivity reactions, including rare cases in which the skin resembles porphyria cutanea tarda (pseudo-porphyria) or epidermolysis bullosa, may occur rarely.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Significant overdosage of the drug may be characterised by drowsiness, heartburn, indigestion, nausea or vomiting. A few patients have experienced seizures, but it is not clear whether these were Naproxen-related or not. It is not known what dose of the drug would be life-threatening. Should a patient ingest a large amount of Dynaflex accidentally or purposefully, the stomach may be emptied and usual supportive measures employed. Animal studies indicate that the prompt administration of Activated Charcoal in adequate amounts would tend to reduce markedly the absorption of the drug. Haemodialysis does not decrease the plasma concentration of Naproxen because of the high degree of protein binding. However, haemodialysis may still be appropriate in a patient with renal failure who has taken Dynaflex.

PACKING/PACK SIZE(S):

DYNAFLEX TABLET 275MG : Blister pack of 50x10's.

DYNAFLEX TABLET 550MG : Blister pack of 50x10's.

JAUHI UBAT DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. (65683-V)

2497, Mk 1, Lorong Perusahaan Baru 5,

Kawasan Perusahaan Perai 3,

13600 Perai, Pulau Pinang, Malaysia.

PI1PT D009-00

Latest Revision: October 2015